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Ysbyty Gwynedd Anaesthetic Department

Opioid Stewardship: Best Practice Guidelines

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1. INTRODUCTION

The opioid epidemic is an international concern. Opioid stewardship aims to improve patient outcomes, reduce opioid-related harm and ensure cost effectiveness whilst providing optimal pain management (Thomson, 2023). All practitioners have a duty to be responsive to public and governmental concern and to be proactive in minimising the risk to public health relating to the misuse of opioids.

Opioids are widely used intraoperatively and used safely and appropriately, are an integral component in postoperative analgesia. Nevertheless, the post-surgical use of opioids is though to be an important source of problems Although is difficult to quantify the problem in the UK, publications indicate the perioperative period is an important source of new persistent opioid use (Soffin et al., 2019).

We have a duty of care to minimise the role anaesthesia, surgery and primary care may have in contributing to the " opioid load " in the community in the UK. It is important that all healthcare professionals involved in surgery and perioperative care work collaboratively to ensure robust opioid stewardship (Varley & Zucherbraun, 2018).

2. POLICY STATEMENT

BCUHB is committed to ensuring opioid stewardship is managed appropriately in adult inpatients and opioids are prescribed administered, monitored, and dispensed appropriately.

3. AIM

The aim of this guidance is to assist Anaesthetists and other healthcare professionals within the department to promote effective opioid stewardship practices that achieve a balance between the administration of sufficient opioid analgesia to facilitate recovery and restoration of function, whilst minimising the risk of opioid-related harm.

4. OBJECTIVES

The principle is to complete a perioperative plan in collaboration with the patient, reflecting their choices and involvement in the surgical journey. All involved health practitioners have the following duties:

1 To ensure all opioids prescribed in the perioperative period are not continued unnecessarily.

2 To ensure patients taking opioids are identified before surgery (Goesling et al., 2015).

3 To identify risk factors for opioid misuse disorder (e.g., anxiety and depression and use of other psychoactive drugs) and ensure patients have access to relevant preoperative and/or subsequent care (Powell et al., 2016).

4 To ensure deprescribing procedures exist at the interface between the hospital and primary care (e.g. communication with the General Practitioner) and that there is effective communication with both the patient and the General Practitioner (GP) and other relevant healthcare practitioners such as nurse prescribers and pharmacists (Neuman et al., 2019).

5 To ensure that chronic post-surgical pain is recognised and treated effectively (Schug et al., 2019)

5. SCOPE

This guidance applies to the following groups of staff who are involved in opioid stewardship in the management of inpatients in acute pain:

- Anaesthetists
- Other doctors and independent prescribers
- Nurses and midwives
- Pharmacists
- Other healthcare providers

The following patient groups are excluded from this guidelines:

- Outpatients

6. ROLES AND RESPONSIBILITIES

Medical, non-medical prescribers, nurses, and pharmacists

1 To ensure all healthcare professionals involved in perioperative care collaborate to provide the highest standards of patient-centred care including opioid stewardship.

2 Healthcare professionals should judiciously use opioids. This means using opioids when needed but stopping when no longer required.

7. MAIN BODY

Preoperative Recommendations

Action: anaesthetists, surgeons, GPs, other healthcare practitioners

1 Preoperative assessment: Patients should be screened for chronic pain and opioid use in the perioperative period. Use of the Oral Morphine Equivalent (OME) should be encouraged and noted.

a. Ideally, it should focus on the biopsychosocial assessment of pain as outlined in the Faculty of Pain Medicine guidelines on the assessment of pain (Faculty of Pain Medicine 2018). This should include a psychosocial assessment and medication history, including psychiatric drugs, analgesics, alcohol, and illicit drugs.

2 Prehabilitation: (optimisation before surgery) ensure the optimal management of preoperative pain including opioid prescribing, psychological preparation education and expectation management.

a. Consideration should be given to reducing preoperative anxiety and catastrophising, as this may improve post-surgical outcomes including pain (Faculty of Pain Medicine, 2024).
b. Preoperative counselling should work collaboratively with the patient and expectation management regarding opioid use and perioperative pain management (Calvin, Bull & Hales, 2019)
c. Patients with complex pain need who may benefit from the Post Anaesthetic Care Unit (PACU) should be identified, and plan formulated

3 Complex cases:

a. For complex pain patients on high dose opioids, the opinion of a pain specialist should be gained prior to surgery.

b. Calculate the Oral Morphine Equivalent Dose (OME), which should be documented in the clinical record (Faculty of Pain Medicine, 2024).Opioid

tolerance and opioid induced hyperalgesia may occur in patients taking opioids.

c. Opioid tolerance (decrease in pharmacological response) and opioid induced hyperalgesia (increase in pain perception) may occur in patients taking opioids. Opioid tolerance is at 60mg of the OME for ≥ 7 days. Avoid escalating opioid doses before surgery (Nielsen et al, 2016).

d. If the enteral route is unavailable immediately after surgery, opioid conversion should be made to parenteral morphine.

e. In patients unsuitable for preoperative opioid de-escalation, opioids taken before surgery should usually be continued throughout the surgical admission.

f. An individualised plan should be made for patients on buprenorphine (sublingual or transdermal patches) or methadone and other specific situations such as pregnancy (Quinlan & Cox, 2017).

g. Opioid-sparing adjuncts should be considered.

4 Perioperative management plan: Formulated with the patient and communicated with the surgical and anaesthetic team.

Intraoperative Recommendations

Action: anaesthetists, surgeons,

1. Intraoperative pain management should include follow the principles of:

a. Promotion of early functional return i.e. drinking eating and mobilisation (Levy et al, 2016).

b. Multimodal analgesia – has been shown to be opioid sparing and supplies superior pain relief (Pitchon et al., 2018)

c. Opioid sparing analgesia techniques and the use of opioids sparing adjuvants are encouraged (Sultan et al., 2017).

2. PROSPECT (Procedure Specific Analgesia Techniques) recommendations for analgesia should be utilised rather than reliance on the WHO analgesic ladder (PROSPECT, 2024)

3. Perioperative pain management techniques must be individualised to the patient. This should be based on shared decision making with the patient, considering the type of surgery, patient comorbidities and pre-existing medicine use.

Postoperative Recommendations

Action: anaesthetists, surgeons, other healthcare practitioners

1. Pain relief should be optimised before leaving recovery. This includes a proper handover of pain management plan to ward, HDU or ITU nurses providing the contact details of staff who could be called for resolving pain management issues (e.g. Acute Pain Team **Bleep 115** or Anaesthetist on call on **Bleep 063**).

2. Functional pain assessment. For patients with complex pain problems, an elevated pain intensity taken in isolation should not be a sole indicator for administering further opioids and should not hinder discharge to the ward. A pain assessment that involves functional assessment (i.e., pain on breathing and movement) along with an awareness of factors such as anxiety that can increase pain perception is recommended.

3. Immediate-release opioids are preferred in the management of post operative pain (to decrease the risk of respiratory impairment and long-term opioid use) when simple analgesics such as paracetamol and Nonsteroidal anti-inflammatory drugs (NSAIDs) are not effective enough to allow the achievement of agreed functional goals.

4. Advice on medicine self-administration: On discharge, advise patients how to self-administer medicines safely, wean analgesics, dispose of unused analgesic medications and the dangers of driving and operating machinery whilst taking opioid medication. Education on the dangers of mixing opioids with alcohol and other illicit drugs, which increases the risk of harm should be provided. To reinforce the message a patient leaflet should be provided (Appendix 2).

5. Local protocols for the prescription of discharge medications after surgery ("TTOs") should be developed to minimise the risk of subsequent inappropriate

opioid use. Ideally this should be managed between the hospital and primary care

6. The hospital discharge letter must state explicitly the recommended opioid dose, amount supplied and planned duration of use.

7. Identification of patients for de-escalation of opioids: Such painful conditions such as osteoarthritis of the knee, may require surgery procedures to treat pain and improve function. Patients with these conditions may be taking opioid analgesia before surgery. These opioids should be gradually withdrawn if possible following surgery.

8. Medication review post discharge: Guidance should be given about a medication review following discharge from hospital. Usually 5 days but no more than 7 days medication should be prescribed.

Recommendations for the Ward

1. Promotion to normal function:

    a. The oral route should be used as soon as possible for medications

    b. It should be realised an increase in pain intensity may be the consequence of surgical complication (e.g. compartment syndrome or anastomotic leak)

    c. Sedation scores should be recorded in addition to the respiratory rate to detect those at risk of opioid induced ventilatory impairment (Frederick et al, 2018).

2. Immediate release opioids are preferred in the management of post operative pain when simple analgesics are insufficient to achieve the analgesic goals. If modified-release opioids (including transdermal) are used, due care should be exercised as they have been associated with harm. The prescribed dose of the immediate-release dose should be age-related (rather than weight) and consider renal function. Liquid oral morphine 10mg/5ml is the preferred option as it is a Schedule 5 drug, which facilitates a timelier administration. Immediate release oxycodone is not recommended as a first-line opioids, as it is a Schedule 2 drug and more labour intensive to administer. However, it may be

useful in elderly patients over 70 years, or in patients with renal failure. Other opioids may be utilised postoperatively in accordance with local policy.

3. When the requirement for analgesics is reduced, a reverse analgesic ladder is recommended: wean opioids first, then stop NSAIDS, then stop paracetamol (Table 1.)

4. The inpatient Acute Pain Service (**Bleep 115**) should be involved of the post-surgical care of opioid tolerant patient (Royal College of Anaesthetist, 2022)
5. Patients on gabapentinoids should be identified and the indication for the gabapentin/pregablin reviewed. Gabapentinoids should be tapered off if no longer required (Medicines and Healthcare Products Regulatory Agency, 2017)

Discharge Planning

Action: anaesthetists, surgeons, GPs, and other healthcare practitioners

1. Safe administration of opioids: Patients should be educated how to self-medicate opioid safely, wean analgesics and dispose of unused medication. Patients should be reminded to store opioids securely which may be liable to misuse. They should also be informed of the dangers of driving and using machinery and provided with a patient leaflet to reinforce the message.

2. A protocol for discharge medication should be used as it reduce subsequent opioid use (Brat et al., 2018).Patients should have access to appropriate simple non-opioid analgesics.

    a. It is preferrable to prescribe opioid and non-opioid analgesics separately on order to allow for dose changes (e.g. paracetamol and codeine instead of co-codamol).
    b. Patients should be encouraged to keep a record of the analgesia taken, as research has shown that this results in better pain relief
    c. New prescriptions of modified-release opioid preparations (including transdermal patches) should be avoided without specialist consultation. If consultation is required it is key to exclude chronic post-surgical pain (Huang et al., 2018).

3. The hospital discharge letter should be available to all healthcare professionals involved in the patients care, including community pharmacists, to avoid an acute prescription of opioids inadvertently becoming a repeat prescription.

The hospital letter must state the recommended opioid dose, amount supplied and planned duration of use.

4. Guidance should be given about medicine review following discharge from hospital:

    a. Usually, 5 days but no more than 7 days (including tramadol) should be prescribed (Brat et al., 2016).

5. Additional recommendations for opioid tolerant patients

    De-escalation of opioids after pain relieving surgery:

    a. For opioid-tolerant patients whose surgery was pain relieving e.g. knee surgery, the discharge letter should provide advice on further weaning of analgesics taken before surgery. The outpatient Chronic Pain Service may be able to assist if difficulties arise (Joel et al., 2019).

Chronic post-surgical pain is defined by IASP (International Association for the Study of Pain) as a clinical discomfort that lasts more than three months post-surgery without other causes of pain such as chronic infection or pain from a chronic condition preceding the surgery. This requires assessment by a GP or the Chronic Pain Team (PainClinicWest@wales.nhs.uk), and to the operating surgeon. Opioids are NOT normally appropriate long-term for chronic post-surgical pain.

Post Discharge Management

Action: anaesthetists, surgeons, GPs, and other healthcare practitioners

1. Educate patients to dispose of unused opioid medication to avoid diversion and misuse. Excess medication should be taken to the community or hospital pharmacy.

2. If a patient not usually on long-term opioids is still taking opioids (including) tramadol 90 days after surgery and still in pain, it should trigger a referral to the outpatient Chronic Pain Service

3. Pain and opioid related admissions should be highlighted to the inpatient Acute Pain Team (Levy & Nicholas, 2019).

Recommendations for best clinical practice

1. All patients undergoing surgery should are at risk of developing persistent postoperative opioid use and opioid-induced ventilatory impairment and may require interventions to mitigate this risk.

2. Consider optimising pre-operative pain and psychological risk factors before surgery, including the weaning of opioids were possible. Ensure realistic expectation of postoperative pain control, both in hospital and following discharge.
3. Provision of opioid analgesia should be guided by functional outcomes, rather than unidimensional pain scores alone.

4. Multimodal analgesia should be optimised, and patients educated about the use of non-pharmacological and non-opioid analgesia to reduce the amount and duration of opioids of opioids required to restore function.

5. Avoid long-acting opioids routinely for acute postoperative pain.

6. A patient-centred approach should be used to limit the number of tablets and duration of usual discharge opioid prescription, typically to less than a week.

7. Automated post-discharge repeat prescriptions for opioids should be avoided. Review patients if more opioids are requested.

8. Hospitals should have strategies to reduce opioid-induced ventilatory impairment. Specifically, all patients receiving post operative opioids must have their level of sedation assessed as appropriate at repeated intervals.

9. Modifiable risk factors have been identified as increasing the risk of opioid-induced ventilatory impairment and persistent postoperative opioid use should be addressed. These factors include medicines with sedative properties; long-acting opioids; and reliance on unidimensional pain scores alone to guide prescribing and titration (Table 2).

10. Patients should be advised on the safe storage and disposal of unused opioids and directed to avoid opioid diversion to other individuals.

Summary

Recommended strategies to reduce opioid related harm include pre-operative assessment of the risk of persistent postoperative opioid use, use of an assessment of patient function rather than unidimensional pain scores alone to guide adequacy of analgesia, avoidance of long-acting (modified-release) and transdermal patches) opioid formulations and combination analgesics, limiting the number of tablets prescribed at discharge, providing deprescribing advice, avoidance of automatic prescription refills, safe disposal of unused medicines; reducing the risk of diversion. and better education of health professions, patients, and carers. Improved prescribing practices may reduce the risk of postoperative opioid related harm in adults (Table 3).

Table 1: Reverse pain ladder to promote postoperative analgesic deprescribing

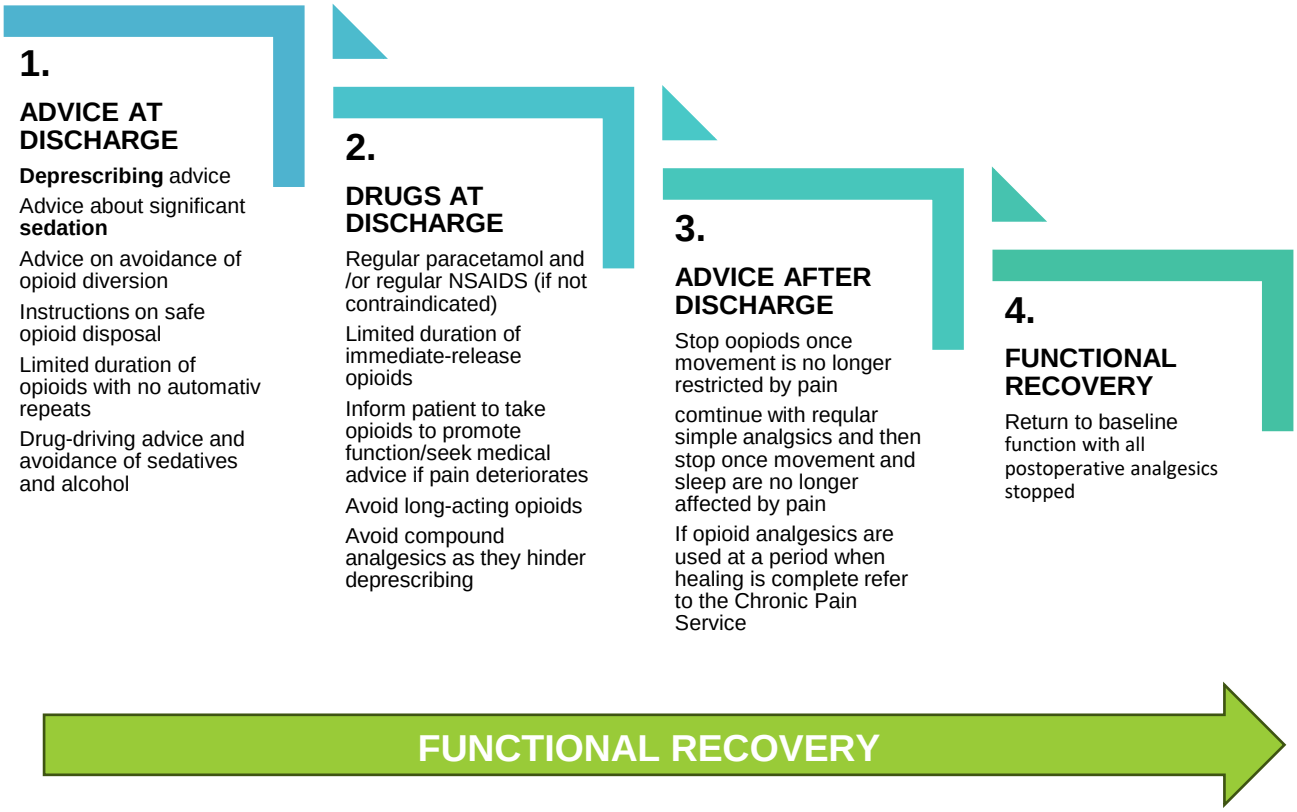
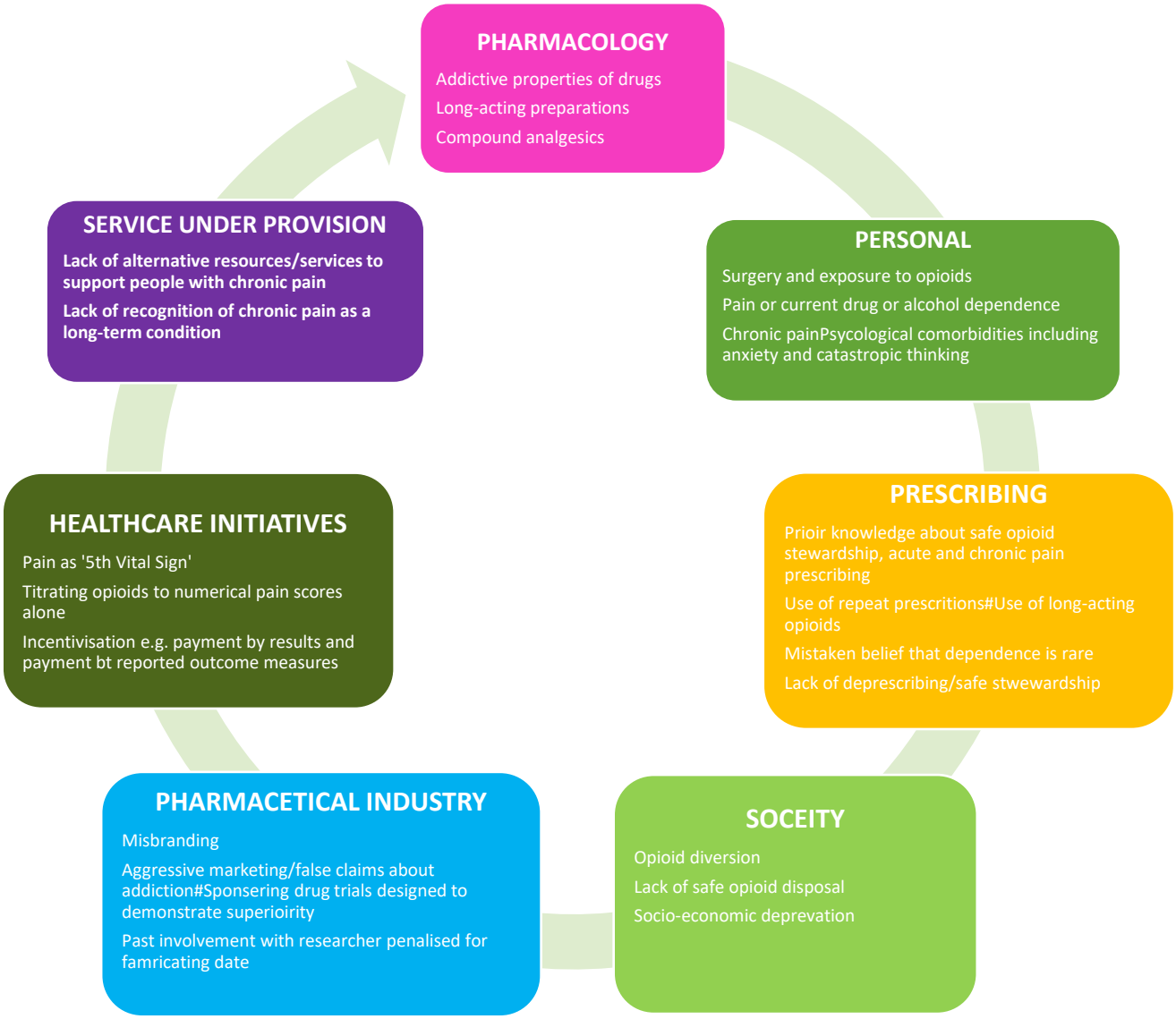


Table 2: Modifiable risk-factors and recommendations for persistent postoperative Opioid Use (PPOU)

Setting	Modifiable Risk Factors	Recommendations
Pre-assessment clinic	Pre-operative opioids	Patients taking long-term are more likely to develop PPOU, wean or taper opioids before surgery
	Psychological comorbidities	Psychological interventions may reduce pre-operative anxiety, depression, and catastrophic thinking, thus decreasing postoperative pain and opioid requirements
	Unrealistic expectations	Education patients and carers about pain management, including non-pharmacological strategies and safe analgesic use

Postoperative period in hospital	Reliance on unidimensional pain scores only	Use functional outcomes to ensure the analgesic use leads to improved recovery
	Abnormal pain trajectory	Patients whose pain is not following the expected trajectory should be identified early as this may signify postoperative complications, neuropathic pain, or psychological distress
	Reliance on opioid analgesia	Use multimodal analgesia and non-pharmacological techniques for pain relief
	Long-acting opioids	Avoid initiating long-acting opioids formulations. Use short acting opioids as needed only
	Compound opioids	Do not prescribe compound opioids give each drug separately
Preparation for discharge	Over-emphasis on opioids for discharge medication	Educate patients and carers to use multimodal analgesia and non-pharmacological techniques for pain relief
	Large number of opioid tablets	Limit the number of tablets given at discharge
	Long duration of discharge opioid prescription	Limit the duration of opioids given at discharge
	Lack of deprescribing advice	Educate patients and carers about reducing analgesia (opioids first)
Post-discharge	Repeat prescription	The risk of PPOU increases with each repeat prescription: review the patient before dispensing more opioids
	Chronic postsurgical pain	Refer to the chronic pain service if pain exceeds expected healing time
	Unsafe storage of opioids at home	Unsecured opioids risk unintended overdose or diversion
	Unsafe disposal of unsafe opioids	Educate patients and carers about the safe disposal of unused opioids

Table 3: Prescribed opioid misuse risk factors



Levy et al., (2021). An international multidisciplinary consensus statement on the prevention of opioid-related harm in adult surgical patients. *Anaesthesia*, *76*(4), 520–536, p523.

8. EQUALITY INCLUDING WELSH LANGUAGE

The equality impact assessment (EQIA) did not identify an adverse impact for any protected group. This best practice guidelines are intended to be a prescribing resource therefore it is not anticipated to impact the use of the Welsh language.

9. WELL-BEING OF FUTURE GENERATIONS

The use of the guideline, for the indication stated in section 1, has no effect on the wellbeing of future generations.

10. ENVIROMENTAL IMPACT ASSESSMENT

An Environment Impact Assessment has not been undertaken as this this opioid stewardship guidance is not anticipated to have an environmental impact.

11. RESOURCES

No additional staffing resources are required. The clinical nurse specialists in pain management and other members of the pain team can support in the implementation of this guideline as part of their usual roles.

12. TRAINING

All healthcare professionals involved in the prescribing and administration of opioids included in this guideline must have received the appropriate education, taking and competence to do so. See Guideline and BCUHB Medicines Policy MM)! and BCUHB Injectable Medicines Policy.

13. IMPLEMENTATION

Successful implementation will require dissemination of information to all applicable staff through the anaesthetic department intranet.

14. FURTHER INFORMATION – CLINICAL DOCUMENTS

Please refer to section 17.

15. AUDIT

Anaesthetists and acute pain nurses will ensure compliance with the guidelines. There is no audit tool available to use.

16. REVIEW

The policy will be reviewed and updated every three years, or sooner if required.

17. REFERENCES

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18. APPENDICES

APPENDIX 1.

Approximate equ-analgesic potencies of opioids for oral administration

	Potency	Equivalent dose to 10mg oral morphine
Codeine Phosphate	0.1	100mg
Dihydrocodeine	0.1	100mg
Hydromorphone	5	2mg
Methadone	*	*
Morphine	1	10mg
Oxycodone	.1.5	6.6mg
Tapentadol	0.4	25mg
Tramadol	0.1	100mg

*The relative potency of **methadone** depends on the starting dose and the duration of administration. Conversions to and from methadone should always be undertaken with specialist advice.*

Transdermal Opioids:

A. Buprenorphine

Transdermal buprenorphine changed at weekly intervals:

	5 micrograms/hr	10 micrograms/hr	20 micrograms/hr
Codeine Phosphate (mg/day)	120mg	240mg	
Morphine sulphate (mg/day)	12mg	24mg	48mg

Transdermal buprenorphine changed every three or four days (twice weekly)

	35 micrograms/hr	52 micrograms/hr	70 micrograms/hr
Morphine sulphate (mg/day)	85mg	126mg	168mg

B. Fentanyl

Fentanyl patch strength (micrograms/hr)	Oral morphine (mg/day)
12	30
25	60
50	120
75	180
100	240

https://fpm.ac.uk/opioids-aware-structured-approach-opioid-prescribing/dose-equivalents-and-changing-opioids

APPENDIX 2.

Patient Information Leaflets

Pain Society (2022) *Managing pain after your surgery*.
https://www.britishpainsociety.org/static/uploads/resources/files/pain_management_print-ready_atwork_2.pdf



Medicines and Healthcare products Regulatory Agency.
Opioids and the risk of addiction
Opioid medicines and the risk of addiction - GOV.UK (www.gov.uk)

Medicines & Healthcare products Regulatory Agency

MHRA

OPIOID MEDICINES AND THE RISK OF ADDICTION

This safety leaflet is to help support you in using opioid medicines safely. Please keep it in a safe place.

Patients, family, friends, and carers can play an important role in the safe use of these medicines and in reducing the risk of harm. Please share this information with them.

What are opioid medicines and what's the risk with them?

Opioids are a type of medicine that help relieve pain. They are very effective over short periods, to relieve moderate to severe pain. They are also sometimes prescribed for long periods, to relieve pain in serious conditions. Examples of opioids include codeine (including co-codamol) tramadol, fentanyl and morphine.

Opioids have a serious risk of **addiction,** especially with long-term use. The person who prescribed an opioid or your pharmacist should explain how long it is safe for you to take your medicine for.

For further information on using opioids safely, and a full list of possible side effects, read the Patient Information Leaflet (PIL) that came with your medicine, and keep it handy.

How can I take my opioid medicine safely?

Only take the medicine as directed.

- **Do not** increase the dose or take an extra dose.
- **Do not** take any other medicines that contain opioids to "top up" your pain relief.
- **Do not** take opioid medicines if you are pregnant without health professional advice.

Do not do any of these without advice from the person who prescribed your medicine, or from a pharmacist. Taking more than you should can lead to overdose.

Babies born to women who took opioids during pregnancy may need to be carefully monitored for withdrawal effects after birth. Talk to your doctor or midwife if you are worried.

If you have any questions about your opioid medicine or side effects, or if you do not understand how to take your medicine, **talk to** the person who prescribed your opioid medicine or to a pharmacist. **Keep talking** to them about your pain – there may be different treatments that can help.

- **Do not allow others** to take any opioid medicines given to you. Your medicine has been prescribed or specifically recommended for you by your doctor or pharmacist and can be dangerous if taken by other people – it could even cause fatal overdose.
- **Always keep medicines out of sight and reach of children.**

How do I know if I'm becoming addicted?

Addiction can happen gradually. It can make you feel that you are no longer in control of how much medicine you need to take or how often you need to take it. You might feel that you need to carry on taking your medicine, even when it doesn't help to relieve your pain.

Medicines & Healthcare products Regulatory Agency

MHRA

MEDDYGINIAETHAU OPIOID A'R RISG O ORDDIBYNIAETH

Mae'r daflen ddiogelwch hon yn helpu eich cynorthwyo chi i ddefnyddio meddyginiaethau opioid yn ddiogel. Cadwch hi mewn lle diogel.

Gall cleifion, teulu, ffrindiau, a gofalwyr chwarae rôl bwysig o ran defnyddio'r meddyginiaethau hyn yn ddiogel ac o ran lleihau'r risg o niwed. Rhannwch y wybodaeth hon gyda nhw.

Beth yw meddyginiaethau opioid a beth yw'r risg gyda nhw?

Mae opioidau yn fath o feddyginiaeth sy'n helpu i leddfu poen. Maen nhw'n effeithiol iawn dros gyfnodau byr, i leddfu poen cymedrol i ddifrifol. Maen nhw weithiau'n cael eu rhagnodi am gyfnodau maith, i leddfu poen yn achos cyflyrau difrifol. Mae enghreifftiau o opioidau'n cynnwys codein (yn cynnwys co-codamol) tramadol, fentanyl a morffin.

Mae gan opioidau risg ddifrifol o **orddibyniaeth,** yn enwedig yn achos defnydd hirdymor. Dylai'r unigolyn a wnaeth ragnodi opioid neu'ch fferyllydd egluro pa mor hir y mae hi'n ddiogel i chi gymryd eich meddyginiaeth.

I gael rhagor o wybodaeth am ddefnyddio opioidau'n ddiogel, a rhestr lawn o sgil-effeithiau posibl, darllenwch y Daflen Wybodaeth i Gleifion (PIL) a ddaeth gyda'ch meddyginiaeth, a'i chadw wrth law.

Sut gallaf i gymryd fy meddyginiaeth opioid yn ddiogel?

Cymerwch y feddyginiaeth yn unol â'r cyfarwyddiadau'n unig.

- **Peidiwch â** chynyddu'r ddos neu gymryd dos ychwanegol.
- **Peidiwch â** chymryd unrhyw feddyginiaethau eraill sy'n cynnwys opioidau i "ychwanegu" at eich cyffur lleddfu poen.
- **Peidiwch â** chymryd meddyginiaethau opioid os ydych yn feichiog heb gael cyngor gan weithiwr iechyd proffesiynol.

Peidiwch â gwneud unrhyw un o'r rhain heb gael cyngor gan yr unigolyn a wnaeth ragnodi eich meddyginiaeth, neu gan fferyllydd. Gall cymryd mwy na y dylech eu cymryd arwain at orddos.

Efallai y bydd angen **monitro babanod a enir i ferched a gymerodd opioidau** yn ystod beichiogrwydd yn ofalus am effeithiau diddyfnu ar ôl yr enedigaeth. Siaradwch â'ch meddyg neu'ch bydwraig os ydych yn bryderus.

Os oes gennych unrhyw gwestiynau ynghylch eich meddyginiaeth opioid neu sgil-effeithiau, neu os nad ydych yn deall sut i gymryd eich meddyginiaeth, **siaradwch â'r** unigolyn a wnaeth ragnodi eich meddyginiaeth opioid neu gyda fferyllydd. **Daliwch i siarad** â nhw am eich poen – fe allai fod gwahanol driniaethau a allai helpu.

- **Peidiwch â gadael i eraill** gymryd unrhyw feddyginiaethau opioid a roddwyd i chi. Mae'ch meddyginiaeth wedi cael ei rhagnodi neu ei hargymell yn benodol i chi gan eich meddyg neu'ch fferyllydd a gall fod yn beryglus os bydd pobl eraill yn ei chymryd – gallai hyd yn oed achosi gorddos angheuol.
- **Cadwch feddyginiaethau o olwg a chyrraedd plant bob amser.**

Sut ydw i'n gwybod os ydw i'n mynd yn orddibynnol?

Gall gorddibyniaeth ddigwydd yn raddol. Gall wneud i chi deimlo nad ydych bellach mewn rheolaeth o ran faint o feddyginiaeth y mae angen i chi ei chymryd nac o ran pa mor aml y mae angen i chi ei